

Header only workbook reconciliation

The supplied 25240-R Database.xlsx contains a 101-column data-sheet header and no populated patient rows; Codebook contains 82 nonempty rows. It is a proposed collection schema, not additional observations. Its mrn field must become a coded patient ID before analytic export.

The workbook preop_tct field is a candidate source for verified baseline TCT. Keep CCT, apical thickness and TCT distinct. The workbook post-op_cdva diopeters header has a unit error: CDVA uses visual-acuity notation or logMAR, never dioptres. Date_of_sx D/M/Y must be parsed with the verified day-month-year convention. The Codebook's preop_cdva and postop_cdva names differ from the displayed headers and require an explicit crosswalk.

Extended topographic indices, aberrometry, densitometry, vectors and haze-resolution fields are optional secondary collections. Their presence in a template does not make them available, validated or part of the primary predictor set.

Provenance and extraction standards

Retain a source hash, source row, exact field, raw value, clean value, rule version, timestamp and reviewer for every mapped observation. Prefer a dated operative report for procedure verification and a dated vision/refraction record for CDVA. A disagreement among sources requires clinical adjudication; no global rule can make one note authoritative for every field.

Automated note extraction must record the quoted evidence location, laterality, date, negation and uncertainty. A mention of haze, progression or surgery without a positive finding and relevant chronology does not establish its presence.

